

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

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Finding Reasoning Paths :

- 1 . **Impaired social interaction , inattention , impulsivity , forgetfulness , and hyperactivity** â Suggestive of a behavioral or neurodevelopmental disorder (e.g ., ADHD).
- 2 . **History of disruptive behavior , including physical aggression (e.g ., hitting)** â May point to a condition involving impulsivity and aggression (e.g ., ADHD or Conduct Disorder).
- 3 . **Family history of major depressive disorder (MDD) in the mother and Graves ' disease in the father** â Raises the possibility of genetic or neurobiological factors influencing behavior (e.g ., neurodevelopmental disorders).
- 4 . **Abnormal physical examination** â No abnormalities found , so physical causes are less likely .
- 5 . **Hyperactivity and poor attention** â Could suggest ADHD or other neurodevelopmental disorders requiring behavioral intervention .

Reasoning Process :

1 . **Symptoms Analysis** :

- The child exhibits a pattern of symptoms consistent with **ADHD**, including inattention , hyperactivity , impulsivity , and poor social interactions . The history of disruptive behavior and inappropriate physical aggression (hitting others) further supports the diagnosis of ADHD with symptoms of impulsivity and aggression .
- ADHD is a neurodevelopmental disorder commonly diagnosed in children , often presenting with inattention , hyperactivity , and impulsivity .

2 . **Exclusion of Other Causes** :

- The absence of abnormalities on the physical examination rules out physical or medical conditions (e.g ., neurological disorders , genetic syndromes).
- There is no evidence of a genetic metabolic disorder (e.g ., tripeptidyl peptidase I deficiency) based on the provided information .
- The family history of major depressive disorder and Graves ' disease does not directly suggest a genetic syndrome (e.g ., MELAS syndrome , grapelaw heterochromia). These are unrelated to the child 's symptoms .

3 . **Treatment Consideration** :

- ADHD is typically treated with a combination of behavior therapy and , if necessary , medication (e.g ., stimulants like methylphenidate or amphetamines).
- Behavior therapy is the cornerstone of treatment for ADHD , focusing on teaching skills to manage symptoms , improve social interactions , and reduce disruptive behavior .

4 . **Exclusion of Other Medications** :

- Citalopram is an antidepressant , not a first-line treatment for ADHD . Similarly , haloperidol is an antipsychotic used for behavioral disorders but is not indicated for ADHD .
- The provided medications (e.g ., haloperidol , zimeldine , citalopram) are not appropriate for this case .

Conclusion :

The most appropriate next step in treatment is **behavior therapy** to address the child 's behavioral and social challenges associated with **ADHD** .

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<answer>

Behavior therapy

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